

1. Administrative & Study Identification

Full Study Title: A Phase 1/2 Open-label Study to Evaluate the Safety and Efficacy of MK-1200 in Participants With Advanced Solid Tumors **Short Title/Acronym:** MK-1200-002 **Protocol Number:** MK-1200-002 **NCT Number:** NCT06242691 **Sponsor Name:** Merck Sharp & Dohme LLC **Investigational Product:** MK-1200 (Biological) / Antiemetic **Phase of Development:** Phase 1 / Phase 2 **Medical Monitor:** Merck Sharp & Dohme LLC Clinical Director

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety, tolerability, and efficacy of MK-1200 monotherapy. Part 1 aims to determine the maximum tolerated dose (MTD). Part 2 aims to evaluate the safety and efficacy of MK-1200 at 2 different doses. **Secondary Objectives:** To evaluate Progression-Free Survival (PFS) and Duration of Response (DOR) as assessed by the investigator and Blinded Independent Central Review (BICR) per RECIST 1.1.

2.2 Background & Rationale To address the critical unmet medical need in participants with advanced/metastatic gastric/gastroesophageal junction (GEJ) cancer, esophageal cancer, biliary tract cancer, and pancreatic ductal adenocarcinoma who have received, or been intolerant to, all treatments known to confer clinical benefit. The study explores MK-1200 as a viable monotherapy option to provide clinical benefit in these heavily pretreated solid tumors.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose escalation and expansion cohorts) **Blinding:** Open-label **Randomization:** N/A (or sequential dose cohort assignment for Part 1; assigned to 2 doses for Part 2)

3.2 Planned Enrollment & Duration Target \$N\$: 13 enrolled participants (Actual) **Number of Sites:** Global/Regional multicenter breakdown **Estimated Study Start:** 28-Feb-2024 **Estimated Primary Completion:** 17-Jun-2025

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Confirmed advanced (unresectable and/or metastatic) solid tumor: gastric cancer (including GEJ),

esophageal cancer, biliary tract cancer, or pancreatic ductal adenocarcinoma. 2. Received and progressed on or after 1 or 2 prior lines of therapy. 3. Participants who experienced Adverse Events (AEs) due to previous anticancer therapies must have recovered to $\leq$ Grade 1 or baseline. HIV, HBV, and HCV-infected participants are eligible if viral loads are well-controlled/undetectable on therapy.

4.2 Exclusion Criteria 1. Known additional malignancy that is progressing or has required active treatment within the past 2 years. 2. Known active central nervous system (CNS) metastases and/or carcinomatous meningitis. 3. Active severe digestive disease, active infection requiring systemic therapy, or have not adequately recovered from major surgery.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: MK-1200 administered at ascending doses in Part 1 to establish MTD, and at 2 distinct defined dosages in Part 2. **Route of Administration:** Intravenous (Biologic infusion) **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or study withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard antiemetics are specified as part of the treatment protocol to manage potential side effects. Standard care for co-morbidities. **Prohibited:** Prior systemic anticancer therapy including investigational agents within 4 weeks before study intervention. Prior radiotherapy within 2 weeks of the start of study intervention.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Imaging (Baseline), Viral Load Labs
Baseline	Day 0	First Dose (MK-1200), Vital Signs, Safety Labs
Interim	Per Cycle	Safety Labs, Efficacy Assessment (RECIST 1.1 Tumor Imaging), DLT monitoring (Part 1)
End of Study	PD/Toxicity	Final Vital Signs, Exit Interview, Follow-up for Survival

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Identification of the Maximum Tolerated Dose (MTD), Dose-Limiting Toxicities (DLTs), and overall safety profile. **Measurement Method:** Clinical assessment of adverse events graded by CTCAE.

7.2 Secondary & Exploratory Endpoints Progression-Free Survival (PFS): Defined as the time from randomization/enrollment to the first documented Progressive Disease (PD) ($\geq 20\%$ increase in sum of diameters of target lesions plus absolute increase of ≥ 5 mm, or new lesions) or death due to any cause. **Duration of Response (DOR):** Defined as the time from the first documented evidence of a Complete Response (CR) or Partial Response (PR) until PD or death, evaluated per RECIST 1.1 by both the investigator and BICR.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of treatment-emergent adverse events throughout the trial and recovery periods. **Serious Adverse Events (SAEs):** Specific monitoring for Dose-Limiting Toxicities (DLTs) during Part 1 to inform dose selection for Part 2. **Data Monitoring Committee (DMC):** Internal safety review committee overseeing dose-escalation progression and safety signals.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving at least one dose of MK-1200) and Efficacy/Intent-to-Treat (ITT) Population for tumor responses. **Sample Size Justification:** Small targeted cohort ($N=13$) indicative of an early-phase (Phase 1/2) dose-escalation/expansion design aiming to identify MTD. **Handling of Missing Data:** Standard survival analysis methodologies, utilizing right-censoring at the last evaluable tumor assessment for PFS and DOR endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote clinical monitoring by the sponsor. **Audit Trail:** Secure eCRF entry, radiographic data stored and evaluated by Blinded Independent Central Review (BICR).

11. Study Identifiers & Governance

Primary ID: MK-1200-002 **Registry Number:** NCT06242691 **Sponsor/Lead Organization:** Merck Sharp & Dohme LLC **Study Title:** Safety and Efficacy of MK-1200 in Participants With Advanced Solid Tumors (MK-1200-002) **Official Regulatory Title:** A Phase 1/2 Open-label Study to Evaluate the Safety and Efficacy of MK-1200 in Participants With Advanced Solid Tumors

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Solid Tumors (Gastric, Esophageal, Biliary Tract, Pancreatic) **Diagnosis Threshold:** Confirmed advanced (unresectable and/or metastatic) progressing after 1-2 prior lines of therapy **Medical Methodology:** Anti-neoplastic Biological Therapy **Optimization Target:** Maximum Tolerated Dose (MTD) discovery and objective tumor response (CR/PR)

13. Study Procedures & Methodology

Management Pathway: Stepwise dose-escalation (Part 1) followed by dose-expansion (Part 2). **Education Protocol (HCP):** Investigator training on RECIST 1.1 criteria, DLT reporting, and biologic administration. **Education Protocol (Patient):** Thorough informed consent addressing experimental Phase 1/2 safety risks. **Quality Audit Mechanism:** Centralized independent imaging review (BICR) for objective response confirmation.

14. Observation & Follow-Up Schedule

Total Duration: From first enrollment (Feb 2024) to primary completion (Jun 2025) **Onsite Visit Cadence:** Aligned with MK-1200 dosing cycles and RECIST 1.1 tumor imaging intervals. **Remote Monitoring Frequency:** Inter-cycle safety and survival tracking. **Checklist Review Interval:** Regular interval scans for Progression-Free Survival (PFS) calculation.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				